

THE ALBERTA DISABILITY SYSTEM BREAKDOWN

April 2026 Report Series

INTERGENERATIONAL IMPACT

How Alberta's Disability System Compounds Across Generations

A Structural Analysis

All claims sourced to the Information Asymmetry Report (April 2026) and primary government documentation. Free to share.

Disability in Alberta is treated administratively as if it begins at the moment of diagnosis and ends at the moment of program exit. Both edges are wrong. The lived reality of disabled families in this province is that the system's failures at each life stage create the conditions for the next generation's encounter with the same system — and that the cost of those compounding failures is borne, generation after generation, by the families inside them.

DISABILITY DOES NOT BEGIN WHERE THE SYSTEM RECOGNIZES IT, AND IT DOES NOT END WHERE A SINGLE PROGRAM ENDS. The compounding mechanism documented below operates across every transition point — pediatric to adult care, FSCD to PDD, AISH to ADAP, working life to succession — and accumulates with each generation that is not given the information needed to interrupt it.

THE THREE-GENERATION PATTERN

The compounding pattern documented in disabled Alberta families is not unique to any one household. It is structural. The version below is composite — drawn from the documented patterns this campaign has gathered across many families, and grounded in the program data and primary sources cited in the Information Asymmetry Report.

Generation One — Acquired Disability, Information Withheld

A grandparent enters the workforce in the 1960s, 1970s, or 1980s. They are exposed at home or at work to substances whose long-term effects are documented in the medical literature of the time but are not communicated to the exposed population. The science on asbestos was clear by the 1960s. The science on lead was clear by the 1970s. The science on PFAS was clear by the 1980s and 1990s. By their fifties, they begin to develop chronic conditions — autoimmune, cardiac, kidney, joint. Each is treated separately because the connecting framework is not part of the clinical picture they are presented with. They retire on a CPP and small private pension below the poverty line, with no public acknowledgment that the institutional information system failed them.

Generation Two — Inherited Risk, Recognized Late

Their child grows up with no warning that their parent's conditions were environmentally produced and may carry epigenetic transmission. The child enters adulthood with vulnerabilities they cannot name and a family history they cannot interpret. Diagnoses arrive in their thirties or forties — autoimmune, neurodevelopmental, mental health, chronic pain — often after years of being told their symptoms are within normal variation. They apply for AISH. The application takes months. The medical adjudicator applies criteria that have not been published. The Citizens Appeal Panel overturned approximately one in three AISH eligibility decisions on appeal in the period for which records are available. By the time supports stabilize, this person is also a parent — and what they have learned navigating their own disability is now needed to navigate their child's.

Generation Three — Diagnosed Late, Aged Out of Intervention

The Alberta Disability System Breakdown · Intergenerational Impact · April 2026 · Free to Share

Their child shows developmental difference at two or three. The family doctor says “some children develop later” — accurate within the constraints of generalist practice, incomplete in the way the report names. By the time a developmental specialist is consulted (multi-month to multi-year wait), the early intervention window has closed. FSCD is applied for. The Inclusion Alberta survey of 540 families across 79 of 87 Alberta electoral districts (January 2025) documents an average wait of three years for full FSCD services. The child reaches age 18 facing the FSCD-to-PDD cliff — a separate program, separate criteria, no continuity of service. If the child’s disability is not intellectual-developmental, no provincial adult support program is available to them at all.

THE COMPOUNDING

At every stage, the family’s capacity to advocate is reduced by the same system they are advocating within.

The grandparent’s estate cannot be planned around disability succession because no one told them to plan for one. The parent cannot fully advocate for the child because the parent’s own AISH file is being reviewed under the AISH-to-ADAP transition, effective July 1, 2026, by a Medical Review Panel using criteria that have not been published, with the independent appeal pathway eliminated by Bill 12. The child’s transition into adulthood encounters the same recognition gap that delayed their parent’s diagnosis a generation earlier.

The province possesses information at each stage — about exposure risks, about diagnostic pathways, about program criteria, about succession planning — that, if communicated to families during the years it would matter, would change the outcome. The information is not communicated. The compounding occurs. The next generation arrives at the same system and is met with the same silence.

NONE OF THIS IS A NATURAL HARDSHIP. EACH STAGE IS A POLICY OUTCOME WITH A POLICY ALTERNATIVE. The province has the operational capacity to publish the information that would interrupt the compounding pattern, and has chosen not to. That choice is what this report names.

*For the full evidence base — Charter, CRPD, and Alberta Bill of Rights argument with footnotes and Sources index — see the **Information Asymmetry Report** (49 pages, April 2026) and its 12-page **Plain Language Companion**, available at albertadisabilitysystembreakdown.netlify.app.*